

SUBJECTIVE:

Patient reports having just fallen off their motorbike at an estimated speed of 80 MPH approximately 15 minutes prior to arrival. Patient verbalizes generalized body pain, noting significant discomfort in the left lower extremity, right wrist, and anterior chest wall. Patient denies initial loss of consciousness but reports feeling dizzy, nauseous, and disoriented to time since the incident. Patient recalls hitting the ground hard and sliding for a distance. Patient is unable to bear weight on the left leg. No pre-existing medical conditions are reported, and no known drug allergies.

OBJECTIVE:

- * General: Patient is a male/female appearing acutely distressed, pale, and diaphoretic. C-spine precautions initiated immediately with cervical collar in place.
- * Vital Signs: BP 90/60 mmHg, HR 120 bpm, RR 24 bpm, SpO2 94% on room air, Temp 36.5 C.
- * Head/Neck: Scalp laceration noted on the right parietal region, actively oozing. Tenderness over cervical spinous processes with C-spine immobilized. Pupils equal and reactive to light. GCS 14 (E4V4M6, disoriented to time).
- * Chest: Bilateral abrasions and ecchymosis to the anterior chest wall. Tenderness to palpation along the right anterior ribs. Breath sounds diminished in the right lower lung field.
- * Abdomen: Distended and firm, with guarding and diffuse tenderness to palpation, more pronounced in the left upper quadrant. Significant road rash across the abdomen.
- * Extremities: Gross deformity, open fracture with exposed bone fragments, and significant swelling noted in the left mid-tibia/fibula. Multiple deep lacerations and extensive road rash on bilateral upper and lower extremities. Swelling and tenderness over the right wrist.
- * Pelvis: Appears unstable to gentle rock.
- * Neurological: No focal motor or sensory deficits noted in extremities at this time, limited by pain.

ASSESSMENT:

- * Primary Diagnosis: Polytrauma secondary to high-speed motorcycle collision. Probable hemorrhagic shock.
- * Differential Diagnosis:
 - * Traumatic brain injury (concussion, epidural/subdural hematoma)
 - * Cervical spine fracture/dislocation with potential spinal cord injury
 - * Pneumothorax, hemothorax, pulmonary contusion, rib fractures
 - * Splenic laceration/rupture, hepatic laceration, retroperitoneal hemorrhage
 - * Pelvic fractures
 - * Open tibia/fibula fracture, right wrist fracture, other extremity fractures
 - * Vascular injury (e.g., femoral artery)
 - * Severe soft tissue injuries and extensive road rash

PLAN:

- * Tests:
 - * STAT trauma labs: CBC, CMP, Type & Crossmatch, Coagulation panel, Lactate, ABG, Urinalysis.
 - * Imaging: Pan-CT scan (Head, Cervical Spine, Chest, Abdomen/Pelvis) with contrast. FAST exam in the trauma bay. X-rays of left tibia/fibula and right wrist.
- * Medications:
 - * IV fluids: Two large bore IVs established, bolus 1-2 liters of warmed crystalloid for hypotension.
 - * Pain management: IV Fentanyl 50-100 mcg as needed.
 - * Tetanus prophylaxis.
 - * Broad-spectrum IV antibiotics for open fracture and lacerations.
- * Patient Advice:
 - * Maintain NPO status.
 - * Strict adherence to C-spine precautions; do not move head or neck.
 - * Explanation of urgent need for diagnostic imaging and potential surgical interventions.
- * Follow-up Instructions:
 - * Admission to trauma center ICU for continuous monitoring and stabilization.
 - * Immediate consults to Trauma Surgery, Orthopedic Surgery, Neurosurgery, and Interventional Radiology.
 - * Prepare for potential emergency operative intervention for hemorrhage control, open fracture debridement, and internal fixation.